# Arrhythmias & Cardiac Electrophysiology

## Page 1: Supraventricular Tachycardias & AV Nodal Reentry

### AVNRT, AVRT, and Orthodromic vs Antidromic Conduction

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High-Yield Clinical Concepts & Board Review Points:

- **AVNRT involves dual AV nodal pathways: a slow pathway with short refractory period and a fast pathway with long**
- **Vagal maneuvers and IV Adenosine transiently block the AV node to terminate reentrant SVTs.**
- **Wolff-Parkinson-White (WPW) syndrome features an accessory pathway (Bundle of Kent) producing a short PR int**

CLINICAL PEARL & EXAM TRAP:

**In WPW with pre-excited atrial fibrillation, AV nodal blockers (adenosine, beta-blockers, verapamil, digoxin) are strictly C**

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## Page 2: Ventricular Tachycardia & Channelopathies

**Monomorphic vs Polymorphic VT and Torsades de Pointes**

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### High-Yield Clinical Concepts & Board Review Points:

- **Sustained monomorphic VT in patients with prior myocardial infarction is almost always due to scar-related macro**
- **Torsades de Pointes occurs in the setting of prolonged QT interval (>500 ms) and is treated acutely with IV Magnes**
- **Brugada syndrome: Autosomal dominant SCN5A mutation causing coved ST elevation in V1-V3 with risk of sudde**

CLINICAL PEARL & EXAM TRAP:

**Synchronized cardioversion is used for hemodynamically unstable VT with a pulse; defibrillation (unsynchronized) is mar**